



AMERICA'S ESSENTIAL HOSPITALS

February 23, 2026

Thomas Keane, MD, MBA
Assistant Secretary for Technology Policy, National Coordinator for Health Information
Technology
U.S. Department of Health and Human Services
Mary E. Switzer Building, Mail Stop: 7033A
330 C St. SW
Washington, DC 20201

**Ref: Request for Information: Accelerating the Adoption and Use of Artificial
Intelligence as Part of Clinical Care**

Dear Assistant Secretary Keane:

On behalf of our nearly 400 member hospitals that serve patients who face barriers to care, we write to share how the Department of Health and Human Services (HHS) can accelerate adoption of responsible artificial intelligence (AI) to support clinical care for all. Essential hospitals are committed to finding innovative ways to improve patient outcomes, and so we look forward to continuing to work with HHS to ensure appropriate regulations and adequate reimbursement for emerging AI technologies.

Since 1981, America's Essential Hospitals has advanced policies and programs that promote health and access to health care. We support our members with advocacy, policy development, research, education, and leadership development. Communities depend on essential hospitals for care across the continuum, health care workforce training, research, public health, and other services. Supported by Essential Hospitals Institute, the association's research and education arm, essential hospitals innovate and adapt to lead all of health care toward better outcomes and value.

The mission of essential hospitals closely aligns with President Trump's vision to make all Americans healthy. Essential hospitals are committed to serving people in all communities that need access to quality care. Although essential hospitals account for only 6% of acute-care hospitals nationwide, in 2023 they provided 29% of the nation's charity care. About three-quarters of the patients our members serve are uninsured or enrolled in Medicaid or Medicare.¹ In addition, nearly two-thirds of essential hospitals provide services to rural patients and

¹ Miu R, Kelly K, Nelb R. *Essential Data 2025: Our Hospitals, Our Patients—Results of America's Essential Hospitals 2023 Annual Member Characteristics Survey*. America's Essential Hospitals. November 2025. essentialdata.info. Accessed Feb. 19, 2026.

communities.² To meet the needs of all patients, essential hospitals constantly engage in robust initiatives to improve quality and access, including efforts to combat chronic health conditions, all while lowering health care costs and spending.

For the past several years, America's Essential Hospitals has committed to supporting member hospitals that harness the power of AI to improve health outcomes for all. In 2024, hospital leaders developed policy principles to help ensure that AI tools can improve access to quality care for underserved populations without unintended consequences.³ Additionally, we convene a monthly interest group for member leaders who currently leverage AI to advance health outcomes, enhance operational efficiency, support financial sustainability, and foster trust.

Based on feedback from our members that are developing, deploying, and evaluating AI best practices and solutions, we offer the following comments on how HHS can leverage regulation, reimbursement, and research to accelerate the adoption of responsible AI at essential hospitals.

Regulatory Tools

- 1. Policies on AI use in health care should clarify the roles of state and federal laws and regulations. Further, policies must not conflict with requirements from other regulatory bodies, such as accrediting organizations.**

As HHS develops regulations governing AI in health care, the agency must consider the interaction between existing federal and state laws and accreditations governing the provision of health care to minimize additional compliance costs.

In states with existing AI laws, essential hospitals have invested significant time and financial resources implementing AI tools and modifying their workflows to use the tools effectively and legally. New federal regulations will require a deeper analysis of both federal and state laws and integration of their requirements. While essential hospitals will comply with all applicable regulations, HHS also should require vendors to ensure their products comply with federal and state law in a timely fashion. Health care providers should not be held accountable when vendors cannot comply.

Federal rulemaking must balance the benefits of regulation and standardization with the required resources for compliance. System modification at essential hospitals is expensive, as is staff training to correctly implement AI tools as regulations change. Frequent regulations will stymie AI adoption at essential hospitals.

Further, hospitals must meet numerous health care facility accreditation requirements and comply with laws safeguarding patients' health care information. Federal AI regulations must not conflict with these requirements. However, federal AI regulations that align with accreditation requirements could benefit all stakeholders.

² America's Essential Hospitals. Policy Brief: Essential Hospitals Ensure Access to Care in Rural Areas. March 2025. <https://essentialhospitals.org/wp-content/uploads/2025/03/2025-Access-to-Care-inRural-Areas-Brief.pdf>. Accessed Jan. 28, 2026.

³ Artificial Intelligence Policy Principles. America's Essential Hospitals. <https://essentialhospitals.org/wp-content/uploads/2026/02/2024-March-AI-Principles.pdf>. Accessed Feb. 12, 2026.

2. HHS should ensure vendors provide additional transparency and testing to help hospital leaders make more informed decisions about AI solutions.

The proliferation of AI tools and rapid updates make selecting the right AI solution difficult. Essential hospitals need transparent, standardized information about AI tools, including development, evaluation data, data provenance, and return on investment, to enable comparison and make informed decisions.

After AI tool and vendor selection, essential hospitals must conduct extensive testing before training staff and implementing the tool, which can be costly and challenging. Essential hospital leaders find it difficult to assess the validity of AI models and tools across clinical settings and populations before committing to a product and testing it internally. Rather than having individual hospitals and health systems conduct these tests, vendors should be required to test these models in a competitive environment and conduct formal adversarial testing before their products are declared ready for health care providers. In addition, vendors should offer flexible, short-term contracting approaches.

While all AI solutions must be piloted before full implementation, HHS should require vendors to test their products in various health care settings and ensure interoperability with other health care technology vendors. This would reduce testing burden on health care providers and accelerate AI adoption.

3. HHS should implement technologies and related standards to safeguard against automating bias in AI systems.

If AI tools are not appropriately trained to include the complex patients that essential hospitals serve, they may provide incorrect answers when diagnosing and treating them in real-world care settings. AI tools deployed in clinical settings should be rigorously trained on representative data and designed to perform safely and effectively for the populations that our members serve. We therefore urge HHS to establish guidelines on mitigating all forms of data bias and create strong AI training standards. HHS also should define appropriate human oversight and accountability structures to monitor the ethical use of AI tools.

Reimbursement Challenges

1. HHS should limit the ability of private insurers to use AI tools to limit care.

Private insurers have rapidly deployed AI tools in claims processing, prior authorization automation, and credentialing. These technologies can inappropriately restrict or delay access to care. As these automated systems create new payment barriers, essential hospitals are increasingly forced to divert limited resources away from patient care and toward adopting their own AI tools to ensure that claims are processed and paid. For example, many essential hospitals now rely on AI-driven claims review systems to prevent wrongful denials and payment delays. Meanwhile, insurers continue to invest in increasingly sophisticated AI designed to scrutinize, slow, or deny claims. Because insurers have significantly larger profit margins than essential hospitals, they can buy the latest tools at a higher volume and faster rate. Therefore, HHS should act to ensure private insurers do not employ AI to boost their profits at the expense of care provision.

2. HHS must address the financial barriers for essential hospitals to adopt and implement AI solutions.

In 2023, members of America's Essential Hospitals had an aggregate operating margin of -7.1%, more than three times lower than the aggregate operating margins for all other hospitals (-2.3%).⁴ These financial strains limit essential hospitals' ability to invest in and scale AI infrastructure and solutions. The high cost of AI tools limits hospitals' ability to implement multiple AI solutions at once, i.e., separate AI solutions in patient care, purchasing, and claims processing. The cost also hampers a hospital's ability to scale AI solutions systemwide.

HHS could mitigate these financial barriers by providing enhanced payments to support AI adoption at essential hospitals through Section 1115 demonstration waivers or other Medicaid or Medicare payment authorities. Supporting the development of AI tools at essential hospitals also could help ensure that the benefits of this technology reach patients who need them most.

Research and Development Opportunities

Essential hospitals are actively testing new AI innovations and hope to continue working with HHS to share best practices for using AI to benefit patients. Some early adopters report improved diagnostic accuracy, optimized scheduling, targeted risk-stratification for readmissions, and reduced staff burnout. However, our members also recognize the necessity of tailored solutions for the high-need patients that they serve.

In addition to testing specific AI clinical applications, there is also a need to research best practices for establishing AI governance frameworks in health systems. Many essential hospitals lack the dedicated expertise, governance structures, and peer networks to translate AI innovations into practice. They face emerging policy and regulatory developments and mounting concerns about validity for complex populations. Thus, essential hospitals need tailored, actionable guidance on AI governance, such as a framework or best practices for implementing AI solutions and oversight structures. HHS could accelerate research on best practices for implementing AI tools for safety net populations by incorporating research and evaluation into targeted investments in essential hospitals' ability to adopt AI technologies.

America's Essential Hospitals appreciates the opportunity to submit these comments. If you have questions, please contact Director of Policy Robert Nelb, MPH, at 202-585-0127 or rnelb@essentialhospitals.org.

Sincerely,



Jennifer DeCubellis (Feb 19, 2026 19:10:38 EST)

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President and CEO

⁴ Miu R, Kelly K, Nelb R. *Essential Data 2025: Our Hospitals, Our Patients—Results of America's Essential Hospitals 2023 Annual Member Characteristics Survey*. America's Essential Hospitals. November 2025. essentialdata.info. Accessed Feb. 19, 2026.